料號	NDDUP05DMY00000
產品中文名	
產品英文名	Budida Soft Capsules 0.5mg
展開尺寸	150mmx440mm
印刷顏色	■ K100
規格	
版號	DUP05DMY-1_01
確認日期	
備註	

10mm

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DUP05DMY-1_01

Budida Soft Capsules 0.5mg

Dutasteride
0.5mg

10mm

QUALITATIVE AND QUANTITATIVE COMPOSITION
Each capsule for oral use contains 0.5 mg dutasteride (*see section List of Excipients*).

PHARMACEUTICAL FORM
Capsules: an opaque, light yellow, and oblong soft gelatin capsule, contains homogeneous and clear liquid.

INDICATIONS
Monotherapy
Dutasteride is indicated for the treatment and control of symptomatic benign prostatic hyperplasia (BPH) in men with an enlarged prostate to improve symptoms, reduce the risk of acute urinary retention, and reduce the risk of the need for BPH-related surgery.

Combination With Alpha-Blocker
Dutasteride in combination with the alpha-blocker tamsulosin is indicated for the treatment of symptomatic BPH in men with an enlarged prostate.

DOSAGE AND ADMINISTRATION
Adult males (including elderly)
Capsules should be swallowed whole and not chewed or opened, as contact with the capsule contents may result in irritation of the oropharyngeal mucosa.

Dutasteride may be taken with or without food.

The recommended dose of Dutasteride is one capsule (0.5 mg) taken orally once a day.

Although an improvement may be observed at an early stage, treatment for at least 6 months may be necessary in order to assess objectively whether a satisfactory response to the treatment can be achieved.

For the treatment of BPH, Dutasteride can be administered alone or in combination with the alpha-blocker tamsulosin (0.4mg).

Renal impairment
The effect of renal impairment on dutasteride pharmacokinetics has not been studied. However, no adjustment in dosage is anticipated for patients with renal impairment.

Hepatic impairment
The effect of hepatic impairment on dutasteride pharmacokinetics has not been studied so caution should be used in patients with mild to moderate hepatic impairment. In patients with severe hepatic impairment, the use of dutasteride is contraindicated.

CONTRAINDICATIONS
Dutasteride is contraindicated in patients with known hypersensitivity to dutasteride, other 5-alpha reductase inhibitors, or any component of the preparation.
Dutasteride is contraindicated for use in women and children and adolescents.
Dutasteride is contraindicated in patients with severe hepatic impairment.

WARNINGS AND PRECAUTIONS FOR USE
General
Lower urinary tract symptoms of BPH can be indicative of other urological diseases, including prostate cancer. Patients should be assessed to rule out other urological diseases prior to treatment with Dutasteride. Patients with a large residual urinary volume and/or severely diminished urinary flow may not be good candidates for 5 alpha-reductase inhibitor therapy and should be carefully monitored for obstructive uropathy.

Blood donation
Men being treated with dutasteride should not donate blood until at least 6 months have passed following their last dose. The purpose of this deferred period is to prevent administration of dutasteride to a pregnant female transfusion recipient.

Prostate cancer
No causal relationship between Dutasteride and high grade prostate cancer has been established. The clinical significance of the numerical imbalance is unknown. Men taking Dutasteride should be regularly evaluated for prostate cancer risk including PSA testing.

Prostate specific antigen (PSA)
Serum prostate-specific antigen (PSA) concentration is an important component of the screening process to detect prostate cancer.

Dutasteride causes a decrease in mean serum PSA levels by approximately 50% after 6 months of treatment.

Patients receiving Dutasteride should have a new PSA baseline established after 6 months of treatment with Dutasteride. It is recommended to monitor PSA values regularly thereafter. Any confirmed increase from lowest PSA level while on Dutasteride may signal the presence of prostate cancer or noncompliance to therapy with Dutasteride and should be carefully evaluated, even if those values are still within the normal range for men not taking a 5-ARI. In the interpretation of a PSA value for a patient taking Dutasteride, previous PSA values should be sought for comparison.

Treatment with Dutasteride does not interfere with the use of PSA as a tool to assist in the diagnosis of prostate cancer after a new baseline has been established.

Total serum PSA levels return to baseline within 6 months of discontinuing treatment.

The ratio of free to total PSA remains constant even under the influence of Dutasteride. If clinicians elect to use percent-free PSA as an aid in the detection of prostate cancer in men undergoing Dutasteride therapy, no adjustment to its value is necessary.

Digital rectal examination, as well as other evaluations for prostate cancer, should be performed on patients prior to initiating therapy with Dutasteride and periodically thereafter.

Increased Risk of High-Grade Prostate Cancer
In men aged 50 to 75 years with a prior negative biopsy for prostate cancer and a baseline PSA between 2.5 ng/mL and 10.0 ng/mL taking AVODART in the 4-year Reduction by Dutasteride of Prostate Cancer Events (REDUCE) trial, there was an increased incidence of Gleason score 8-10 prostate cancer compared with men taking placebo (AVODART 1.0% versus placebo 0.5%). In a 7-year placebo controlled clinical trial with another 5-alpha reductase inhibitor (finasteride 5 mg, PROSCAR), similar results for Gleason score 8- 10 prostate cancer were observed (finasteride 1.8% versus placebo 1.1%).

5-alpha reductase inhibitors may increase the risk of development of highgrade prostate cancer. Whether the effect of 5-alpha reductase inhibitors to reduce prostate volume, or study-related factors, impacted the results of these studies has not been established.

Cardiovascular adverse events
No causal relationship between Dutasteride (alone or in combination with an alpha blocker) and cardiac failure has been established.

Breast cancer
Prescribers should instruct their patients to promptly report any changes in their breast tissue such as lumps or nipple discharge.

Leaking capsules
Dutasteride is absorbed through the skin, therefore women, children and adolescents must avoid contact with leaking capsules. If contact is made with leaking capsules the contact area should be washed immediately with soap and water.

Hepatic impairment
The effect of hepatic impairment on dutasteride pharmacokinetics has not been studied. Because dutasteride is extensively metabolised and has a half-life of 3 to 5 weeks, caution should be used in the administration of dutasteride to patients with liver disease.

DRUG INTERACTIONS
Dutasteride is metabolised by human cytochrome P450 isoenzyme CYP3A4. Therefore blood concentrations of dutasteride may increase in the presence of inhibitors of CYP3A4.

Decrease in clearance of dutasteride when co-adminstered with the CYP3A4 inhibitors verapamil (37%) and diltiazem (44%). In contrast no decrease in clearance when amlodipine, another calcium channel antagonist, was coadminstered with dutasteride.

A decrease in clearance and subsequent increase in exposure to dutasteride, in the presence of CYP3A4 inhibitors, is unlikely to be clinically significant due to the wide margin of safety (up to 10 times the recommended dose has been given to patients for up to six months); therefore no dose adjustment is necessary.

Dutasteride is not metabolized by human cytochrome P450 isoenzymes CYP1A2, CYP2A6, CYP2E1, CYP2C8, CYP2C9, CYP2C19, CYP2B6, and CYP2D6.

Dutasteride neither inhibits human cytochrome P450 drug-metabolizing enzymes nor induces cytochrome P450 isozymes CYP1A, CYP2B and CYP3A in rats and dogs.

Dutasteride does not displace warfarin, diazepam, acenocoumorol, phenprocoumon, or phenytoin from plasma protein, nor do these model compounds displace dustasteride. Compounds that have been tested for drug interactions in man include tamsulosin, terazosin, warfarin, digoxin, and cholestyramine, and no clinically significant pharmacokinetic or pharmacodynamic interactions have been observed.

No clinically significant adverse interactions were observed when dutasteride was co-administered with anti- hyperlipidemics, angiotensin-converting enzyme (ACE) inhibitors, beta-adrenergic blocking agents, calcium channel blockers, corticosteroids, diuretics, nonsteroidal anti-inflammatory drugs (NSAIDs), phosphodiesterase Type V inhibitors, and quinolone antibiotics.

PREGNANCY AND LACTATION
Fertility
The clinical significance of dutasteride's effect on semen characteristics for an individual patient’s fertility is not known.

Pregnancy

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Dutasteride is contraindicated for use by women. Dutasteride has not been studied in women because pre- clinical data suggests that the suppression of circulating levels of dihydrotestosterone may inhibit the development of the external genital organs in a male foetus carried by a woman exposed to dutasteride.

Lactation
It is not known whether dutasteride is excreted in breast milk.

EFFECTS ON ABILITY TO DRIVE OR OPERATE MACHINES
Based on the pharmacokinetic and pharmacodynamic properties of dutasteride treatment with dutasteride would not be expected to interfere with the ability to drive or operate machinery.

ADVERSE REACTIONS
Adverse drug reactions are listed below by system organ class and frequency. Frequencies are defined as: very common, common, uncommon, rare and very rare including isolated reports.

Immune system disorders
Very rare: Allergic reactions, including rash, pruritus, urticaria, localised oedema, and angioedema.

Psychiatric disorders
Very rare: Depressed mood

Skin and subcutaneous tissue disorders
Rare: Alopecia (primarily body hair loss), Hypertrichosis

Reproductive system and breast disorders
Very rare: Testicular pain and testicular swelling

OVERDOSE
There is no specific antidote for dutasteride therefore, in cases of suspected overdosage, symptomatic and supportive treatment should be given as appropriate.

PHARMACOLOGICAL PROPERTIES
Pharmacodynamics
Pharmacotherapeutic group: testosterone-5- alpha-reductase inhibitors.
ATC code: G04C B02.

Dutasteride is a dual inhibitor of 5 alpha reductase. It inhibits both type 1 and type 2, 5 alpha reductase isoenzymes, which are responsible for the conversion of testosterone to dihydrotestosterone (DHT). DHT is the androgen primarily responsible for hyperplasia of glandular prostatic tissue.

Pharmacokinetics
Absorption
Dutasteride is administered orally in solution as a soft gelatin capsule. Following administration of a single 0.5mg dose, peak serum concentrations of dutasteride occur within 1 to 3 hours.

Absolute bioavailability in man is approximately 60% relative to a 2-hour intravenous infusion. The bioavailability of dutasteride is not affected by food.

Distribution
Pharmacokinetic data following single and repeat oral doses show that dutasteride has a large volume of distribution (300 to 500 L). Dutasteride is highly bound to plasma proteins (>99.5%).

Following daily dosing, dutasteride serum concentrations achieve 65% of steady state concentration after 1 month and approximately 90% after 3 months. Steady state serum concentrations (Css) of approximately 40 nanograms/mL are achieved after 6 months of dosing 0.5 mg once a day. Similarly to serum, dutasteride concentrations in semen achieved steady state at 6 months. After 52 weeks of therapy, semen dutasteride concentrations averaged 3.4 nanograms/mL (range 0.4 to 14 nanograms/mL). Dutasteride partitioning from serum into semen averaged 11.5%.

Biotransformation
In vitro, dutasteride is metabolised by the human cytochrome P450 isoenzyme CYP3A4 to two minor monohydroxylated metabolites, but it is not metabolized by CYP1A2, CYP2A6, CYP2E1, CYP2C8, CYP2C9, CYP2C19, CYP2B6 or CYP2D6.

In human serum, following dosing to steady state, unchanged dutasteride, 3 major metabolites (4'- hydroxydutasteride, 1,2-dihydrodutasteride and 6- hydroxydutasteride) and 2 minor metabolites (6,4'- dihydroxydutasteride and 15- hydroxydutasteride), as assessed by mass spectrometric response, have been detected. The five human serum metabolites of dutasteride have been detected in rat serum, however the stereochemistry of the hydroxyl additions at the 6 and 15 positions in the human and rat metabolites is not known.

Elimination
Dutasteride is extensively metabolised. Following oral dosing of dutasteride 0.5mg/day to steady state in humans, 1.0% to 15.4% (mean of 5.4%) of the administered dose is excreted as dutasteride in the faeces. The remainder is excreted in the faeces as 4 major metabolites comprising 39%, 21%, 7%, and 7% each of drug- related material and 6 minor metabolites (less than 5% each).

Only trace amounts of unchanged dutasteride (less than 0.1% of the dose) are detected in human urine.

At therapeutic concentrations, the terminal half-life of dutasteride is 3 to 5 weeks.

Serum concentrations remain detectable (greater than 0.1 ng/mL) for up to 4 to 6 months after discontinuation of treatment.

Linearity/non-linearity
Dutasteride pharmacokinetics can be described as first order absorption process and two parallel elimination pathways, one saturable (concentration-dependent) and one non-saturable (concentration independent).

At low serum concentrations (less than 3 nanograms/mL), dutasteride is cleared rapidly by both the concentration-dependent and concentration-independent elimination pathways. Single doses of 5 mg or less showed evidence of rapid clearance and a short half-life of 3 to 9 days.

At serum concentrations, greater than 3 nanograms/mL, dutasteride is cleared slowly (0.35 to 0.58 L/h) primarily by linear, non-saturable elimination with terminal half-life of 3 to 5 weeks. At therapeutic concentrations, following repeat dosing of 0.5 mg/day, the slower clearance dominates and the total clearance is linear and concentration-independent.

Elderly
No dutasteride dose-adjustment based on age is necessary.

Renal impairment
No adjustment in dosage is anticipated for patients with renal impairment.

Hepatic impairment
The effect on the pharmacokinetics of dutasteride in hepatic impairment has not been studied.

PHARMACEUTICAL PARTICULARS
List of Excipients
Capsule contents: monodiglycerides of caprylic/capric acid; butylated hydroxytoluene
Capsule shell: gelatin; glycerin; titanium dioxide; yellow ferric oxide

Incompatibilities
Not applicable.

Special Precautions for Storage
Do not store above 30°C.

Shelf Life
The expiry date is indicated on the packaging.

Pack Size
3 x 10's capsules

Nature and Contents of Container
PVC/PVDC film/Aluminum blisters

Instructions for Use/Handling
Capsules should be swallowed whole and not chewed or opened, as contact with the capsule contents may results in irritation of the oropharyngeal mucosa.

Dutasteride may be taken with or without food.

Dutasteride is absorbed through the skin, therefore women and children and adolescents must avoid contact with leaking capsules. If contact is made with leaking capsules the contact area should be washed immediately with soap and water. Women who are pregnant or may be pregnant should not handle dutasteride soft gelatin capsules because of the possibility of absorption of dutasteride and the potential risk of a foetal anomaly to a male foetus.

PRODUCT REGISTRATION HOLDER
GK Bio International Sdn. Bhd.
No. 43-1, Jalan PJU 5/21, The Strand, Kota Damansara, 47810 Petaling Jaya, Selangor.

IMPORTER
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No. 43-1, Jalan PJU 5/21, The Strand, Kota Damansara, 47810 Petaling Jaya, Selangor.

MANUFACTURER
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